

Intercostal Chest Drain Check list and Report

Place undertaken ☐ treatment room ☐ bedside ☐ radiology ☐ theatres ☐ ICU ☐ endoscopy ☐ ED

Name _____
DoB _____
Hospital number _____
(please affix patient label and confirm identity)

Pre procedure Checklist (Sign in)

Name: _____ Name : _____

Have all team members introduced themselves and role : ☐ Y ☐ N (ICU, unconscious)

Indication: ☐ Air ☐ Fluid ☐ Both

Radiology reviewed: ☐ CT ☐ CXR

Confirm side of procedure: ☐ Left ☐ Right

Observations: BP: ____ SpO2: ____ FiO2: ____ HR: ____

Patient's coagulation and medication checked:

If on anticoagulants please consult the UHL

Anticoagulation Bridging policy on Insite

☐ Yes ☐ No Platelets ____ PT ____

Consent: ☐ Written ☐ Verbal ☐ Part IV

Thoracic US for Fluid done: ☐ Y ☐ N ☐ N/A

Thoracic US findings: ☐ Echoic ☐ anechoic

Effusion depth (cm) ____ Other findings: _____

☐ Realtime US ☐ immediate US marking

Procedural Checklist and Report

Aseptic technique: Sterile ☐ Gown ☐ Gloves
☐ At least two applications of chloroprep
☐ Sterile field protected by drapes

STOP if unable to aspirate Air or Fluid with local anaesthetic

Side: ☐ left ☐ right Site: _____

Lignocaine ☐ 1% ☐ 2% ____ (mls)

Adrenaline present with lignocaine ☐ Y ☐ N

Concentration: _____

Fluid appearance: _____

Samples: ☐ Biochemistry ☐ Cytology ☐ MC&S

Amount drained initially: _____ (mls)

Drain Size: ____ Secured: ☐ Suture ☐ Dressing

Closing mattress (>size 18) placed ☐ Y ☐ N

Complications: Pain (0-10) ____ Other: _____

Guidewire removed: ☐ Y ☐ N ☐ N/A



Post Procedural Checklist (Sign Out)

Order Post procedure CXR and handover for review: ☐ Y ☐ N

Start Chest Drain Chart ☐ Y Prescribe analgesia ☐ Y

Information leaflet on chest drain care given to patient and explained: ☐ Y ☐ N (ICU)

Ensure specimens correctly labelled: ☐ Y ☐ N ☐ N/a

Observations: BP: ____ SpO2: ____ FiO2: ____ HR: ____

Confirm instructions on fluid drainage to nursing staff: ☐ Y

- Drain no more than 1500mls of fluid at any one time.
- Stop drainage by closing tap if patient develops pain, coughing, breathlessness or 1500mls drained for 2 hours then reopen.

Confirm frequency of observations: ☐ every 15 mins for 1 hour then hourly for 2 hours then 4 hourly.

Have all items of stock running low (<3 remaining) been ordered urgently: ☐ Y ☐ N or N/A

Are there procedural problems that need follow-up: ☐ Y ☐ N

If anticoagulation has been stopped please undertake a 'bleeding risk assessment' on when anticoagulation should recommence – consult the UHL Anticoagulation Bridging Policy on Insite
(document plan below)

Recommence anticoagulation plan:

Doctor inserting drain: _____ Grade: _____

Signature: _____ Date: ____/____/____

Supervised: ☐ Y ☐ N Assistant ☐ Y ☐ N

Name: _____ Grade: _____ Signature: _____